

Transurethral resection of a bladder tumour (TURBT)

A transurethral resection of a bladder tumour (TURBT) is an operation to take a biopsy or remove cancer from inside the bladder.

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What is a transurethral resection of a bladder tumour (TURBT)?

A TURBT is used to take a [biopsy](#) or remove cancer from inside [the bladder](#). Transurethral means through the urethra. A TURBT:

- is the main treatment for non-muscle-invasive [bladder cancer](#)
- may be part of diagnosis and treatment for muscle-invasive bladder cancer.

What happens before a TURBT?

Before your operation, you will have a pre-operative assessment. You will be allocated a keyworker who will be able to answer any questions you have. You usually have tests to check your general health. These can include:

- [blood tests](#)
- a recording of your heart, called an [ECG](#)
- a chest [x-ray](#).

The nurse or doctor may ask you for a sample of your urine (pee) to check for any infection. If you have a urine infection, your doctor will give you antibiotics to treat it before surgery.

The nurse or doctor will explain the operation and tell you if there is anything you should do before the operation. This can include instructions about medications or eating and drinking.

This is a good time to ask any questions, or to talk about any worries you may have.

We have more information about [preparing for surgery](#).

On the day of your operation

A member of the surgical team will talk to you about having a general anaesthetic or a spinal anaesthetic:

- For a general anaesthetic you are given medicines to make you sleep through the operation.
- For a spinal anaesthetic, the doctor injects a drug through a thin needle into your back. This numbs the nerves from the waist down. You will be awake during the operation, but you will not feel anything.

You will be asked to wear a hospital gown and elastic stockings called TED stockings during the operation. The stockings help to prevent blood clots in your legs. You will be asked to wear these for some time after the operation.

Having a TURBT for bladder cancer

During a TURBT, the surgeon puts a thin, rigid tube called a cystoscope through the urethra into the bladder. They remove the tumour (or tumours) using surgical instruments that are passed through the cystoscope.

They may use a mild electrical current to stop any bleeding. This is called cauterisation. Any samples removed for biopsy will be sent away to the laboratory.

Having a TURBT takes about 15 to 90 minutes depending on the size and number of tumours in the bladder.

Sometimes, [blue-light cystoscopy or narrow-band imaging](#) are used during surgery to help your doctor remove the tumour (or tumours).

Once the cystoscope is removed a tube called a catheter is put through the urethra into the bladder. This drains urine (pee) from the bladder into a collection bag. This will stay in place until your urine has less blood in it.

What happens after a TURBT?

After your surgery, the nurses will encourage you to start moving as soon as possible. They will also encourage you to do regular leg movements and deep breathing exercises. This helps to prevent problems such as chest infections or blood clots.

You will see blood in your urine (pee) at first. This is common and your nurse will monitor it. Drinking fluids will help to flush out your bladder and reduce the risk of getting a urine infection.

Sometimes, a large bag of fluid is connected to the catheter. This flushes out the bladder and helps to stop blood clots forming. Inside the catheter tube is split so fluid can flow into and out of the bladder at the same time through different channels. A drainage bag collects urine and fluid as it flows out of the catheter.

Tell your nurse straight away if:

- you feel your bladder is full
- your tummy feels uncomfortable
- the catheter stops draining.

Your nurse will take the catheter out when your urine has less blood in it. This may feel a little uncomfortable, but it should not be painful. After the nurse removes the catheter, you may find it uncomfortable to pass urine at first, but this will get better. You may also:

- need to pass urine more often
- need to pass urine urgently
- leak urine
- have some blood in your urine.

These symptoms usually get better in 1 to 2 days. Tell your doctor if they do not get better. Drinking at least 2 litres (3½ pints) of fluid every day can help to reduce these symptoms.

When can I go home?

You can usually go home when you can pass urine (pee) without the catheter. This may be on the same day as your operation or 1 to 3 days after. Some people may notice blood in their urine about 10 to 14 days after the surgery. If this bleeding is very heavy, or if you pass blood clots, tell your nurse or doctor straight away.

When you are at home, it is important to follow the advice given by your specialist nurse. You may need to avoid lifting or carrying anything heavy for 2 to 3 weeks. You can try to do some light exercise, such as walking. This can help to build up your energy, so you can slowly get back to your normal activities.

If you [work](#), your doctor or nurse can give you advice about when to go back.

How long do I have to wait for results?

The surgeon sends the tumour (or tumours) they have removed to a laboratory to be looked at under a microscope. This tells your doctor more about the [stage and grade](#) of the cancer.

Getting the results of your TURBT usually takes 2 to 3 weeks. There is support available while you are [waiting for your results](#).

When the results are ready, they are discussed at a [multidisciplinary team \(MDT\)](#) meeting. Your doctor will then tell you if you need further treatment.

Will I have any further treatment?

If you have non-muscle-invasive bladder cancer, you usually have [chemotherapy into the bladder](#) straight after surgery. This is given through the catheter.

You may have another TURBT 3 to 6 weeks after the first one, for the following reasons:

- If you have a high-risk, non-muscle-invasive bladder cancer. This is to make sure all of the tumour has been removed.
- If your surgeon was unable to remove all of the tumour at your first TURBT.
- If samples taken during your first operation did not contain cells from the bladder muscle layer. This is to check the cancer has not started to spread into the muscle.

If you have muscle-invasive bladder cancer, your doctor will talk to you about your treatment options. This is usually an operation to remove the bladder called a [cystectomy](#), or [radical radiotherapy](#).

What are some of the risks of having a TURBT?

Having a TURBT is usually a safe procedure. But there are some possible risks or complications.

Urine infection

Some people may get a urine tract infection (UTI). This can happen in hospital or after you go home. Signs of an infection include:

- pain or burning feeling when you pass urine (pee)
- needing to pass urine more often
- smelly or cloudy urine
- needing to pass urine straight away
- needing to pass urine more often
- blood in your urine
- lower tummy or back pain
- feeling cold, shivery, hot or sweaty.

Tell your doctor if you think you have a urine infection. They can test your urine and give you antibiotics if you need them.

Bleeding

Some people may have bleeding for about 7 to 14 days after the operation. Tell your doctor or specialist nurse straight away if:

- the bleeding becomes heavier
- you see any blood clots in your urine or on toilet paper
- you have difficulty passing urine
- you have tummy pain.

If the bleeding continues, you may need another operation under general anaesthetic to stop it and to remove any clots.

Damage to the bladder

There is a small risk of a hole being made in the bladder during surgery. This is called a perforation.

If this happens, your surgeon will explain what treatment you may need. This may be:

- the catheter is left in the bladder to allow the hole to heal, and is then removed
- an operation to close the hole.

About our information

References



Below is a sample of the sources used in our bladder cancer information. If you would like more information about the sources we use, please contact us at cancerinformationteam@macmillan.org.uk

Mottet N, Bellmunt J, Briers E, et al. Non-muscle-invasive bladder cancer (TaT1 and CIS). European Association of Urology (Internet), 2021. Available from uroweb.org/guideline/non-muscle-invasive-bladder-cancer (accessed September 2021).

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Reviewers



This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer. It has been approved by Senior Medical Editor, [Dr Ursula McGovern](#), Consultant Medical Oncologist.

Our cancer information has been awarded the [PIF TICK](#). Created by the Patient Information Forum, this quality mark shows we meet PIF's 10 criteria for trustworthy health information.

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ⓘ This content is currently being reviewed. New information will be coming soon.

Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

How we can help

Macmillan Support Line

The Macmillan Support Line is a free and confidential service for people living with and affected by cancer. If you need to talk, we'll listen.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help